

Name : **MR. SRI KRISHNA KOLLIPARA**

Age/Gender : 21 years / Male

Sample Type : WB EDTA

Sample ID : 24220622

Client Name : 3KLTVM1831

Ref. Doctor : SELF

Collected : Mar 07, 2025, 02:56 p.m.

Received : Mar 07, 2025, 08:18 p.m.

Reported : Mar 07, 2025, 11:56 p.m.

MEDID : MB1457286



CLINICAL BIOCHEMISTRY

TEST DESCRIPTION
TCS WELLNESS PACKAGE ONSITE

RESULT

UNITS

REFERENCE RANGES

Glycosylated Hemoglobin (GHb/HbA1c)

HbA1c

(Method: HPLC)

5.54

%

< 6.0 : Non Diabetic

6.1 – 6.5 : Prediabetic

6.6 – 7.0 : Good Control

7.1-8.0 : POOR Control

>8.1 : ALERT

Estimated Average Glucose (eAG)

112.30

%

70 - 136

Interpretation:

Excellent Control: 6 to 7 %

Fair to Good Control: 7 to 8 %

Unsatisfactory Control: 8 to 10 %

and Poor Control: More than 10% .

Factors Influencing HbA1c Results: Increased levels: Elevated fetal hemoglobin, chronic renal failure, iron deficiency anemia, splenectomy, heightened serum triglycerides, alcohol consumption, poisoning (Lead, Opiate), and salicylate therapy. **Decreased levels:** are often associated with systemic inflammatory diseases and reduced RBC life span, severe iron deficiency & haemolytic anaemia, chronic renal failure, and liver diseases. Clinical correlation reduced red blood cell lifespan (such as in hemolytic anemia or blood loss), following blood transfusions, during pregnancy, excessive intake of Vitamin E or Vitamin C, and hemoglobinopathies, For HbF > 25% and homozygous hemoglobinopathy, an alternate platform (Fructosamine) is recommended for testing of HbA1c.

- HbA1c, also known as glycosylated hemoglobin or glycated hemoglobin, refers to hemoglobin that has glucose molecules attached. It gives an average of glucose levels in the bloodstream for the preceding 2 to 3 months.
- HbA1c has been endorsed by clinical groups & ADA (American Diabetes Association) guidelines for the diagnosis of diabetes using a cut-off point of 6.5%.
- For diabetic patients achieving treatment objectives, the HbA1c test should be conducted at least biannually. If treatment objectives are not met or if a new regimen is initiated, testing once every 3 months is recommended.
- The HbA1c target for non-pregnant adults is generally set at below 7% to prevent microvascular complications.
- In known diabetic patients, the following values can be considered as a tool for monitoring glycemic control.

Associated tests: HOMA IR index, insulin, C-peptide levels.

END OF REPORT

Dr NEMANI RAMYA
MD BIOCHEMISTRY

This is an electronically authenticated report. Report Printed Date: Mar 08, 2025, 05:16 a.m.

NOTE: Assay results should be correlated clinically with other clinical findings and the total clinical status of the patient.

Name : **MR. SRI KRISHNA KOLLIPARA**

Age/Gender : 21 years / Male

Sample Type : Naf Plasma

Sample ID : 24220621

Client Name : 3KLTVM1831

Ref. Doctor : SELF

Collected : Mar 07, 2025, 02:56 p.m.

Received : Mar 07, 2025, 08:18 p.m.

Reported : Mar 07, 2025, 09:36 p.m.

MEDID : MB1457286



CLINICAL BIOCHEMISTRY

TEST DESCRIPTION	RESULT	UNITS	REFERENCE RANGES
TCS WELLNESS PACKAGE ONSITE			

Glucose - Fasting

Glucose - FBS

(Method: Hexokinase)

86

mg/dL

70 - 105

Interpretation:

Blood glucose test may be used to detect high blood glucose (hyperglycemia) and low blood glucose (hypoglycemia). Screen for diabetes in people who are at risk before signs and symptoms are apparent; in some cases, there may be no early signs or symptoms of diabetes. Screening can therefore be useful in helping to identify it and allowing for treatment before the condition worsens or complications arise, Help diagnose diabetes, prediabetes and gestational diabetes, Monitor glucose levels in people diagnosed with diabetes.

END OF REPORT

Dr NEMANI RAMYA
MD BIOCHEMISTRY

This is an electronically authenticated report. Report Printed Date: Mar 08, 2025, 05:16 a.m.

NOTE: Assay results should be correlated clinically with other clinical findings and the total clinical status of the patient.

Name : **MR. SRI KRISHNA KOLLIPARA**

Age/Gender : 21 years / Male

Sample Type : SERUM

Sample ID : MB1457286

Client Name : 3KLTVM1831

Ref. Doctor : SELF

Collected : Mar 07, 2025, 02:56 p.m.

Received : Mar 07, 2025, 10:05 p.m.

Reported : Mar 08, 2025, 12:10 a.m.

MEDID : MB1457286



CLINICAL BIOCHEMISTRY

TEST DESCRIPTION
TCS WELLNESS PACKAGE ONSITE

RESULT

UNITS

REFERENCE RANGES

Thyroid Stimulating Hormone (TSH)

Thyroid Stimulating Hormone (TSH)

(Method: CLIA)

2.58

uIU/mL

0.52-16.0: 1 Day - 30 Days

0.55-7.10: 1 Mon - 5 Yrs

0.37-6.00: 6 Yrs - 18 Yrs

0.35-5.50: 18 Yrs - 55 Yrs

0.50-8.90 : > 55 yrs

Interpretation

Healthcare providers often face patients complaining of symptoms related to thyroidism.

- In some patients with these symptoms. TSH and 14 levels can be within normal limits, which could lead healthcare providers to rule out low thyroid function. However, thyroid metabolism is complex, and each step can be influenced by nutrition, prescription medications, and lifestyle factors. Considering these influences can help practitioners in their understanding of how patients can suffer from symptoms of hypothyroidism even when their blood levels appear normal,

- Evaluating TSH, free T4, free T3, reverse T3, thyroid antibodies, cortisol, vitamin D, ferritin, medication use, and nutritional habits all can help healthcare providers determine the cause of a patient's thyroid symptoms, and an appropriate care plan can be developed.

- Nutritional deficiencies can be corrected, lifestyle habits can be addressed and medication dosages can be adjusted based on a detailed patient history and evaluation of the lab values mentioned previously. If thyroid replacement is necessary, healthcare providers can utilize a commercially available thyroid medication.

END OF REPORT

Dr NEMANI RAMYA
MD BIOCHEMISTRY

This is an electronically authenticated report. Report Printed Date: Mar 08, 2025, 05:16 a.m.

NOTE: Assay results should be correlated clinically with other clinical findings and the total clinical status of the patient.

Name : **MR. SRI KRISHNA KOLLIPARA**

Age/Gender : 21 years / Male

Sample Type : SERUM

Sample ID : MB1457286

Client Name : 3KLTVM1831

Ref. Doctor : SELF

Collected : Mar 07, 2025, 02:56 p.m.

Received : Mar 07, 2025, 10:05 p.m.

Reported : Mar 08, 2025, 03:05 a.m.

MEDID : MB1457286



CLINICAL BIOCHEMISTRY

TEST DESCRIPTION TCS WELLNESS PACKAGE ONSITE	RESULT	UNITS	REFERENCE RANGES
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Kidney Function test

Creatinine (Method: Jaffe Kinetic)	1.11	mg/dL	0.7 - 1.4
Urea (Method: Urease)	20	mg/dL	Upto 50
Blood Urea Nitrogen (BUN) (Method: Calculated)	9.35	mg/dL	7 - 18
Blood Urea Nitrogen (BUN)/Creatinine (Method: Calculated)	8.42	Ratio	6 - 22
Uric Acid (Method: Uricase)	4.58	mg/dL	3.4 - 7.0
Urea/Creatinine (Method: Calculated)	18.02	Ratio	

Interpretation:

Creatinine: Muscles produce creatinine, a waste product, from creatine phosphate, a substance that stores a lot of energy. Unlike urea, the amount of creatinine generated is constant and mostly depends on muscle mass. Age, gender, race, muscularity, exercise, pregnancy, and several other physiological characteristics can all have an impact on serum creatinine levels. Decreased serum Creatinine is associated with increasing Age and poor muscle mass, such as muscular atrophy. Both acute and chronic renal disease and blockage are associated with elevated blood creatinine levels. Creatinine is not an appropriate indicator for identifying kidney disease in its early stages since an increase in blood creatinine is only seen when there is significant nephron damage.

High **Urea, Uric Acid, and Blood Urea Nitrogen (BUN)** could indicate poor renal function, in addition to other etiologies

END OF REPORT

Dr NEMANI RAMYA
MD BIOCHEMISTRY

This is an electronically authenticated report. Report Printed Date: Mar 08, 2025, 05:16 a.m.

NOTE: Assay results should be correlated clinically with other clinical findings and the total clinical status of the patient.



Name : MR. SRI KRISHNA KOLLIPARA

Age/Gender : 21 years / Male

Sample Type : SERUM

Sample ID : MB1457286

Client Name : 3KLTVM1831

Ref. Doctor : SELF

Collected : Mar 07, 2025, 02:56 p.m.

Received : Mar 07, 2025, 10:05 p.m.

Reported : Mar 08, 2025, 04:50 a.m.

MEDID : MB1457286



CLINICAL BIOCHEMISTRY

TEST DESCRIPTION	RESULT	UNITS	REFERENCE RANGES
TCS WELLNESS PACKAGE ONSITE			

Electrolyte Profile

Sodium (Method: ISE)	138.3	mmol/L	135 - 145
Potassium (Method: ISE)	4.4	mmol/L	3.8 - 5.2
Chloride (Method: ISE)	98.5	mmol/L	94 - 108

Interpretation:

Sodium:

Low levels: prolonged vomiting or diarrhea, diminished reabsorption in the kidney, and excessive fluid retention. **Pseudo-hyponatremia** is a laboratory artifact. It is usually caused by hypertriglyceridemia, cholestasis (lipoprotein X), and hyperproteinemia (monoclonal gammopathy, intravenous immunoglobulin [IVIg]). **Diluted sampling** should also be suspected in such cases and confirmed on a repeat fresh sample if indicated clinically.

High levels: excessive fluid loss, high salt intake, and increased kidney reabsorption.

Potassium:

Low levels: reduced intake of dietary potassium or excessive loss of potassium from the body due to diarrhea, prolonged vomiting, or increased renal excretion.

High levels: dehydration or shock, severe burns, hemolysis, diabetic ketoacidosis, and retention of potassium by the kidney. **Pseudohyperkalemia**, which may result from a hemolyzed or aged sample, should always be ruled out by doing a repeat electrolyte estimation on a fresh sample, as clinically indicated.

Chloride:

Low levels are noted in reduced dietary intake, prolonged vomiting, and reduced renal reabsorption, as well as some forms of acidosis and alkalosis.

High levels are found in dehydration, kidney failure, some forms of acidosis, high dietary or parenteral chloride intake, and salicylate poisoning

END OF REPORT

 Dr D ANAMIKA MBBS
 DNB PATHOLOGY

This is an electronically authenticated report. Report Printed Date: Mar 08, 2025, 05:16 a.m.

NOTE: Assay results should be correlated clinically with other clinical findings and the total clinical status of the patient.

Name : **MR. SRI KRISHNA KOLLIPARA**

Age/Gender : 21 years / Male

Sample Type : SERUM

Sample ID : MB1457286

Client Name : 3KLTVM1831

Ref. Doctor : SELF

Collected : Mar 07, 2025, 02:56 p.m.

Received : Mar 07, 2025, 10:05 p.m.

Reported : Mar 08, 2025, 03:03 a.m.

MEDID : MB1457286



CLINICAL BIOCHEMISTRY

TEST DESCRIPTION TCS WELLNESS PACKAGE ONSITE	RESULT	UNITS	REFERENCE RANGES
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Liver Function Profile* - PL01

Bilirubin Total (Method: Diazo Method)	0.85	mg/dL	0 - 1.0
Bilirubin Direct (Method: Diazo method)	0.19	mg/dL	0 - 0.3
Bilirubin Indirect (Method: Calculated)	0.66	mg/dL	0 - 1.0
Alkaline Phosphatase (ALP) (Method: PNPP, AMP Buffer)	58	U/L	50 - 136
Alanine Transaminase (ALT/SGPT) (Method: UV without pyridoxal -5- phosphate)	31	U/L	10 - 50
Aspartate Aminotransferase(AST/SGOT) (Method: IFCC Without Pyridoxal Phosphate)	27	U/L	10 - 50
SGOT / SGPT Ratio (Method: Calculated)	0.87		
Y- Glutamyl Transferase (GGT) (Method: glutamyl-carboxynitroanilide)	12	U/L	8 - 61
Protein Total (Method: Biuret)	7.1	g/dL	6.4 - 8.3
Albumin (Method: Bromocresol Green)	4.4	g/dL	3.5 - 5.4
Globulin (Method: Calculated)	2.70	g/dl	2.5 - 3.5
Albumin/Globulin (Method: Calculated)	1.63	Ratio	1.0 - 2.1

END OF REPORT

Dr NEMANI RAMYA
MD BIOCHEMISTRY

This is an electronically authenticated report. Report Printed Date: Mar 08, 2025, 05:16 a.m.

NOTE: Assay results should be correlated clinically with other clinical findings and the total clinical status of the patient.

Name : **MR. SRI KRISHNA KOLLIPARA**

Age/Gender : 21 years / Male

Sample Type : SERUM

Sample ID : MB1457286

Client Name : 3KLTVM1831

Ref. Doctor : SELF

Collected : Mar 07, 2025, 02:56 p.m.

Received : Mar 07, 2025, 10:05 p.m.

Reported : Mar 08, 2025, 03:04 a.m.

MEDID : MB1457286



CLINICAL BIOCHEMISTRY

TEST DESCRIPTION
TCS WELLNESS PACKAGE ONSITE

RESULT

UNITS

REFERENCE RANGES

Lipid Profile* - PL02

Cholesterol - Total

(Method: Cholesterol Oxidase, Esterase, peroxidase)

171

mg/dL

<200 : Desirable

200-239 : Borderline risk

>240 : High risk

Cholesterol - HDL

(Method: Enzymatic Colorimetric)

47

mg/dL

< 40 : Low

40 - 60 : Optimal

> 60 : Desirable

Cholesterol - LDL

(Method: Enzymatic Colorimetric)

109

mg/dL

< 100 : Normal

100 - 129 : Desirable

130 – 159 : Borderline-High

160 – 189 : High

> 190 : Very High

Cholesterol VLDL

(Method: Calculated)

15

mg/dL

7 - 40

Triglycerides

(Method: Lipase / Glycerol Kinase)

75

mg/dL

< 150 : Normal

150–199 : Borderline-High

200–499 : High

> 500 : Very High

Total cholesterol/HDL Ratio

(Method: Calculated)

3.64

0 - 5.0

Non HDL Cholesterol

(Method: Calculated)

124

mg/dL

Desirable: Less than 130

Above Desirable: 130 - 159

Borderline High: 160 - 189

High: 190 - 219

Very high: > or = 220

LDL / HDL Cholesterol Ratio

(Method: Calculated)

2.32

HDL / LDL Cholesterol Ratio

(Method: Calculated)

0.43

END OF REPORT


Dr NEMANI RAMYA
MD BIOCHEMISTRY

This is an electronically authenticated report. Report Printed Date: Mar 08, 2025, 05:16 a.m.

NOTE: Assay results should be correlated clinically with other clinical findings and the total clinical status of the patient.

Name : **MR. SRI KRISHNA KOLLIPARA**

Age/Gender : 21 years / Male

Sample Type : URINE

Sample ID : MB1457286

Client Name : 3KLTVM1831

Ref. Doctor : SELF

Collected : Mar 07, 2025, 02:56 p.m.

Received : Mar 07, 2025, 10:05 p.m.

Reported : Mar 07, 2025, 10:43 p.m.

MEDID : MB1457286



CLINICAL PATHOLOGY

TEST DESCRIPTION	RESULT	UNITS	REFERENCE RANGES
TCS WELLNESS PACKAGE ONSITE			

Complete Urine Analysis - CUE

PHYSICAL EXAMINATION:

Colour Pale Yellow
Appearance Clear

CHEMICAL EXAMINATION:

Specific gravity (Method: Bromothymol Blue)	1.015	1.000 - 1.030
Reaction (pH) (Method: Bromothymol Blue / Methylred)	7.0	5.0 - 8.5
Proteins (Method: Buffered Tetra Bromophenol blue)	Negative	Negative
Bile salts & Bile pigments	Negative	
Glucose (Method: Glucose Oxidase And Peroxidase Method)	Negative	Negative
Bilirubin (Method: Diazonium Salt Method)	Negative	Negative
Ketones (Method: Nitroprusside Reaction)	Negative	Negative
Blood (Method: Hemo Peroxidase)	Negative	Negative
Urobilinogen (Method: Ehrlich Reaction)	Normal	Normal
Nitrites (Method: Griess Test)	Negative	Negative

Microscopic Examination

PUS(WBC) Cells (Method: Microscopy)	3-4	0 - 5 /HPF
RBC	Nil	NIL
Epithelial Cells	2-3	0 - 5/HPF
Casts	Nil	NIL
Crystals	Nil	Nil
Bacteria	Absent	
Budding Yeast cells	Absent	
Others	Nil	--

END OF REPORT

Dr D ANAMIKA MBBS
DNB PATHOLOGY

Processed At : Thiruvananthapuram, Kerala, India This is an electronically authenticated report. Report Printed Date: Mar 08, 2025, 05:16 a.m.

NOTE: Assay results should be correlated clinically with other clinical findings and the total clinical status of the patient.

Name : **MR. SRI KRISHNA KOLLIPARA**

Age/Gender : 21 years / Male

Sample Type : WB EDTA

Sample ID : 24220622

Client Name : 3KLTVM1831

Ref. Doctor : SELF

Collected : Mar 07, 2025, 02:56 p.m.

Received : Mar 07, 2025, 08:18 p.m.

Reported : Mar 08, 2025, 12:24 a.m.

MEDID : MB1457286



HAEMATOLOGY

TEST DESCRIPTION
TCS WELLNESS PACKAGE ONSITE

RESULT

UNITS

REFERENCE RANGES

Erythrocyte Sedimentation Rate (ESR)

Erythrocyte Sedimentation Rate (ESR)

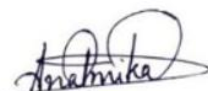
9

mm/Hour

< 10

(Method: Westergren's)

END OF REPORT


Dr D ANAMIKA MBBS
DNB PATHOLOGY

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Name : **MR. SRI KRISHNA KOLLIPARA**

Age/Gender : 21 years / Male

Sample Type : WB EDTA

Sample ID : 24220622

Client Name : 3KLTVM1831

Ref. Doctor : SELF

Collected : Mar 07, 2025, 02:56 p.m.

Received : Mar 07, 2025, 08:18 p.m.

Reported : Mar 07, 2025, 11:52 p.m.

MEDID : MB1457286



HAEMATOLOGY

TEST DESCRIPTION	RESULT	UNITS	REFERENCE RANGES
TCS WELLNESS PACKAGE ONSITE			

Advanced Complete Blood Count with Peripheral Smear Examination

RBC INDICIES

Hemoglobin (Hb)* (Method: Photometry)	15.3	g/dL	13.0 - 18.0
Erythrocyte Count (RBC Count) (Method: Electronic Impedance)	5.0	mil/ μ L	4.5 - 5.5
PCV (Hematocrit) (Method: Calculated)	45.1	%	40 - 54
Red Cell Distribution Width (RDW-SD)	42.9	FL	39 - 46
Mean Corpuscular Volume (MCV)	90.6	fl	83 - 101
Mean Corpuscular Hemoglobin (MCH)	30.8	pg	27 - 32
Mean Corpuscular Hb concentration (MCHC)	34.0	g/dL	31.5 - 34.5
Red Cell Distribution Width (RDW-CV)	12.6	%	11.5 - 14.5

WBC INDICES

White Blood Cells (WBC)	6390	cells/Cumm	4000 - 11000
Neutrophils	44.5	%	40 - 75
Lymphocytes	46.8	%	20 - 40
Eosinophils	4.8	%	0 - 6
Monocytes	3.7	%	2 - 10
Basophils	0.2	%	0 - 1
Absolute Neutrophils Count	2843.55	cells/cumm	2000 - 7000
Absolute Lymphocyte Count	2990.52	cells/cumm	1000 - 3000
Absolute Eosinophil Count	306.72	cells/cumm	50 - 500
Absolute Monocyte Count	236.43	cells/cumm	200 - 800
Absolute Basophil Count	12.78	cells/cumm	0 - 10

PLATELET INDICES

Platelet Count (Method: Electronic Impedance)	3.62	lakh/Cumm	1.50 - 4.50
Mean Platelet Volume (MPV)	8.8	fl	7.5 - 11.5
Platelet Distribution Width (PDW)	10.3	fl	8.3 - 25.0
Platelet large cell ratio (P-LCR)	22.2	fl	11.9 - 66.9
Plateletcrit (PCT)	0.32	%	0.15 - 0.62

Dr D ANAMIKA MBBS
DNB PATHOLOGY

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Name : **MR. SRI KRISHNA KOLLIPARA**

Age/Gender : 21 years / Male

Sample Type : WB EDTA

Sample ID : 24220622

Client Name : 3KLTVM1831

Ref. Doctor : SELF

Collected : Mar 07, 2025, 02:56 p.m.

Received : Mar 07, 2025, 08:18 p.m.

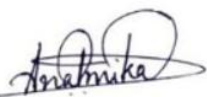
Reported : Mar 07, 2025, 11:52 p.m.

MEDID : MB1457286



HAEMATOLOGY

TEST DESCRIPTION	RESULT	UNITS	REFERENCE RANGES
Peripheral Blood Smear Examination			
RBC Morphology	Normocytic Normochromic		
WBC Morphology	Within Normal Limits		
Platelet	Adequate on smear		
Haemoparasites	Not-Detected		
Impression	Normal Peripheral Smear		
Advise	Kindly correlate clinically		
	END OF REPORT		


Dr D ANAMIKA MBBS
DNB PATHOLOGY

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NOTE: Assay results should be correlated clinically with other clinical findings and the total clinical status of the patient.



Personal Details
UHID: 900206920
PatientID: 900206920
Name: Sri Krishna Kollipara
Age: 21
Gender: Male
Mobile: 0000000000

**Pre-Existing Medical-
Conditions**

Symptoms

Vitals

Measurements
HR: 78 BPM
PR: 134 ms
PD: 112 ms
QRSD: 121 ms
QRS Axis: 99 deg
QT/QTc: 337/384 ms

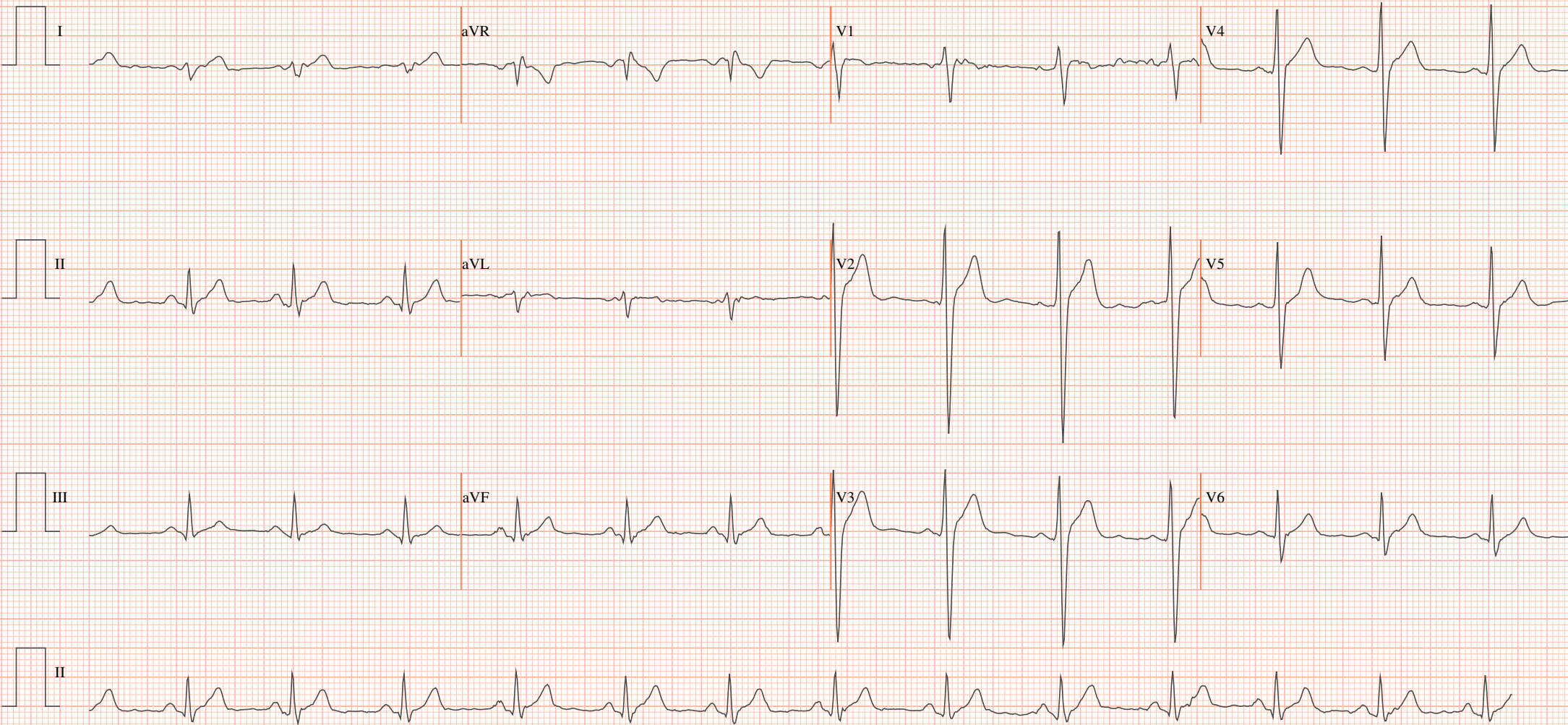
Interpretation

Sinus rhythm
Normal axis
To correlate clinically

Authorized by
Yogesh

Dr.Yogesh Kothari
MD,DNB,FESC,FEP
Reg No- KMC 44065

*This trace is generated by **KardioScreen**; Cloud-Connected, Portable, Digital, 6-12 Lead Scalable ECG Platform from **IMEDRIX***



Speed: 25 mm/sec F: 0.05 - 40 Hz Limb: 10 mm/mV Chest: 10 mm/mV



Vitals Health Assessment

Name:	Sri Krishna Kollipara
--------------	-----------------------

Height (in cm)	169
Weight (in kgs)	62
Systolic BP (mm Hg)	122
Diastolic BP (mm Hg)	76
BMI	21.71